

TEMPOL TABLET

DESCRIPTION: A white, oblong tablet with marking 'DUO 861'.

COMPOSITION: Each tablet contains Paracetamol 500mg.

PHARMACODYNAMICS:

Paracetamol is a para-aminophenol derivative that has analgesic and antipyretic actions similar to those of aspirin, but it has no useful anti-inflammatory properties.

PHARMACOKINETICS:

After oral administration, paracetamol is absorbed rapidly and complete from the small intestine; peak plasma levels occur 30 -120 minutes after administration. Paracetamol is uniformly distributed throughout most body fluids; the apparent volume of distribution is 1 to 1.2 L / kg. Paracetamol can cross the placenta and is excreted in milk. Plasma protein binding is negligible at usual therapeutic concentrations but increases with increasing concentrations.

Paracetamol is metabolized by the hepatic microsomal enzyme system. In adults at therapeutic doses, paracetamol is mainly conjugated with glucuronide (45 – 55%) or sulphate (20 – 30%). A minor proportion (less than 20%) is metabolized to catechol derivatives, and mercapturic acid compounds via oxidation.

Paracetamol is metabolized differently by infants and children compared to adults, the sulphate conjugate being predominant.

Paracetamol is excreted in the urine mainly as the glucuronide and sulfate conjugates. Less than 5% is excreted as unchanged paracetamol with 85 to 90% of the administered dose eliminated in the urine within 24 hours of ingestion. The elimination half-life varies from 1 to 3 hours. Food intake delays paracetamol absorption.

INDICATIONS: Indicated for relief of mild to moderate pain associated with headache, muscular aches, period pain, arthritis, toothache, migraine, cold & flu, tension headache and sinus pain/headache, and for reduction of fever. Indicated as a suitable alternative for whom aspirin is contraindicated, eg. dyspepsia, gastrointestinal ulcers, aspirin sensitivity.

RECOMMENDED DOSAGE:

Adults and children over 12 years: 500 – 1000 mg, every 3 – 4 hours as required. Dosage should not exceed 4 g per day. To be taken with water.

Pediatric (children 7 – 12 years): Dosage is 250 – 500 mg taken every 3 – 4 hours as required. Dosage should not exceed 2 g per day.

ROUTE OF ADMINISTRATION: To be taken orally only.

CONTRAINDICATIONS: Contraindicated to those allergic/ hypersensitive to Paracetamol/ Acetaminophen.

WARNING & PRECAUTIONS:

This preparation contains PARACETAMOL. Do not take any other PARACETAMOL containing medicines at the same time.

Paracetamol should be administered with caution to patients with hepatic or renal dysfunction. Used with caution if restricted salt intake is indicated.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

INTERACTION WITH OTHER MEDICATIONS:

Paracetamol may affect prothrombin time. Anticoagulant dosage may require reduction if paracetamol is prolonged. Anticoagulant dosage may require reduction if Paracetamol medication is prolonged. Paracetamol absorption is increased by drugs which increase gastric emptying, eg metoclopramide, and decreased by drugs which decrease gastric emptying, eg propantheline, antidepressants with anticholinergic properties, narcotic analgesics. Paracetamol may increase chloramphenicol concentrations. The likelihood of paracetamol toxicity may be increased by the concomitant use of enzyme inducing agents such as alcohol or anticonvulsant drugs.

- With metoclopramide – absorption rate of paracetamol increased.
- With pethidine – absorption of paracetamol delayed significantly.
- With propantheline – rate of absorption of paracetamol retarded.
- With warfarin – activity of anticoagulants possibly potentiated.

USE IN PREGNANCY & LACTATION:

Use in pregnancy: Paracetamol can cross the placenta; however, no teratogenic effects have been observed in rats or mice, after doses up to 250 mg / kg.

Use in lactation: Paracetamol is excreted in breast milk. The amount available for ingestion by the infant has been reported variously as less than 0.1% of a single 500 mg dose and as 0.04% to 0.23% of a single 650 mg dose. Maternal ingestion of paracetamol in usual analgesic doses does not appear to present a risk to the nursing infant.

SIDE EFFECTS / ADVERSE EFFECTS: Reports of adverse reactions are rare. Although the following adverse reactions have been reported, a causal relationship to the administration of paracetamol has been neither confirmed nor refuted: dyspepsia, nausea, allergic and haematological reactions. Additionally, cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

SYMPTOMS AND TREATMENT OF OVERDOSE: Accidental overdose is usually rare due to the high therapeutic index of the product. Overdose in children may be related to accidental overdose by an adult. Overdose can result in severe liver damage and sometimes acute renal tubular necrosis. Toxic symptoms include vomiting, abdominal pain, hypotension, sweating, central stimulation with exhilaration and convulsions in children, drowsiness, respiratory depression, cyanosis and coma. In adults, hepatotoxicity may occur after ingestion of a single dose of paracetamol 10 to 15 g (20 to 30 tablets or 10 to 15 times the normal dose); a dose of 25 g (50 tablets) or more is potentially fatal. Symptoms during the first two days of acute poisoning by paracetamol do not reflect the potential seriousness of the intoxication. Major manifestations of liver failure such as jaundice, hypoglycaemia and metabolic acidosis may take at least three days to develop.

Treatment: Prompt treatment is essential even when there are no obvious symptoms. In cases of overdosage, methods of reducing absorption of ingested drug are important. Prompt administration of activated charcoal 50 g in 500 mL of water and 150 mL sorbitol 50% solution by mouth may reduce absorption. It is recommended that intravenous fluids such as Normal Saline be given concurrently. Gastric lavage is indicated if the patient is unwilling or unable to drink an activated charcoal/ sorbitol mixture.

If the history suggests that paracetamol 15 g or more has been ingested, administer the following antidote:

Intravenous acetylcysteine 20%. Administer acetylcysteine immediately without waiting for positive urine test or plasma level results if 8 hours or less since overdose ingestion. Initial dose 150 mg/kg over 15 minutes, followed by continuous infusion of 50 mg/kg in glucose 5% 500 mL over four hours and 100 mg/kg in glucose 5% 1 L over 16 hours. If more than 8 hours have elapsed since the overdosage was taken, the antidote may be less effective.

STORAGE CONDITIONS: Store below 30°C. Protect from light. Keep out of reach of children. *Jauhkan daripada kanak-kanak.*

SHELF LIFE: Please refer to outer package.

PACK SIZE: Pack in box of 50 Blisters x 10 Tabs

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

Lot 2599 Jalan Seruling 59, Kawasan 3,
Taman Klang Jaya, 41200 Klang, Selangor
Darul Ehsan, Malaysia.